

1. Administrative & Study Identification

Full Study Title: ROSETTA CRC-203: A Blinded, Randomized Phase 2/3 Study of Pumitamig in Combination With Chemotherapy Versus Bevacizumab in Combination With Chemotherapy in Participants With Previously Untreated, Unresectable, or Metastatic Colorectal Cancer **Short Title/Acronym:** ROSETTA CRC-203 **Protocol Number:** CA266-0003 **NCT Number:** NCT07221357 **Sponsor Name:** Bristol Myers Squibb **Investigational Product:** Pumitamig + Chemotherapy (FOLFOX, FOLFIRI, or CAPOX) **Phase of Development:** Phase 2, Phase 3 **Trial Status:** Not Yet Recruiting **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: * Phase 2: To evaluate the Objective Response (OR) of pumitamig in combination with FOLFOX/FOLFIRI vs bevacizumab + FOLFOX/FOLFIRI in participants with 1L metastatic colorectal cancer (mCRC). * Phase 3: To compare Progression-Free Survival (PFS) of pumitamig in combination with chemotherapy vs bevacizumab + chemotherapy in participants with 1L mCRC.

Secondary Objectives: To compare Overall Survival (OS); to inform the dose of pumitamig to continue in Phase 3; to characterize pharmacokinetics (PK), immunogenicity, and changes in cancer-related symptoms and Quality of Life (QOL).

2.2 Background & Rationale To address the need for improved first-line therapeutic options in patients with recurrent or metastatic colorectal adenocarcinoma. Research indicates that combining pumitamig with standard chemotherapy may improve clinical outcomes compared to standard FOLFOX or FOLFIRI regimens. This study evaluates the safety and efficacy of the pumitamig-chemotherapy combination against the established standard of bevacizumab plus chemotherapy, particularly in patients without dMMR, MSI-H, and BRAF V600E mutations.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Bevacizumab + Chemotherapy) **Blinding:** Blinded (Double-blind) **Randomization:** Randomized

3.2 Planned Enrollment & Duration Target **\$N\$:** 990 **Number of Sites:** ~143 locations **Estimated Study Start:** March 2026 **Estimated Primary Completion:** February 2030

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Previously untreated, histologically confirmed recurrent or metastatic colorectal adenocarcinoma, not amenable to curative surgery. 3. No known presence of mismatch repair deficient (dMMR) or microsatellite instability-high (MSI-H) colorectal cancer per historical results. 4. No known presence of the BRAF V600E mutation. 5. Measurable disease as defined by Response Evaluation Criteria in Solid Tumors (RECIST) v1.1. 6. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 - 1.

4.2 Exclusion Criteria 1. Untreated known central nervous system (CNS) metastases including brain, leptomeningeal, and/or spinal cord compression. 2. Prior malignancy active within the previous 2 years, except for locally curable cancers that have been apparently cured and are considered low risk of recurrence. 3. Significant cardiovascular disease, such as myocardial infarction, unstable angina, arterial thrombosis, or cerebrovascular accident within 6 months prior to randomization, or uncontrolled hypertension ($\geq 160/100$ mmHg). 4. Prior systemic treatment with an anti-PD-1, anti-PD-L1, anti-PD-L2, CD137 agonists, or anti-CTLA-4 antibody, or any other antibody targeting T cell co-stimulation or checkpoint pathways.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Pumitamid in combination with standard 1L chemotherapy regimens (FOLFOX, FOLFIRI, or CAPOX) administered in continuous treatment cycles (e.g., 14-day or 21-day cycles). **Route of Administration:** Intravenous (IV) and/or Oral (PO), depending on the specific chemotherapy backbone utilized. **Duration of Treatment:** Until disease progression, unacceptable toxicity, withdrawal of consent, or reaching the maximum 5-year study duration.

5.2 Concomitant & Prohibited Therapy Permitted: Routine anti-emetics, supportive care, and inhaled or topical steroids in the absence of active autoimmune disease. **Prohibited:** Systemic treatment with corticosteroids (> 10 mg daily prednisone equivalent) or other immunosuppressive medications within 14 days of randomization; concurrent live vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline RECIST v1.1 Imaging, Labs
Baseline	Day 0	Randomization, First Dose Administration, ECOG Status Check
Interim	Ongoing Cycles	Safety Labs, Chemo/Pumitamidg dosing, Efficacy Assessment (RECIST)
End of Study	Variable	Final Tumor Imaging, Safety Follow-up, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: - Phase 2: Objective Response (OR) of pumitamidg + chemotherapy vs bevacizumab + chemotherapy. - Phase 3: Progression-Free Survival (PFS) of pumitamidg + chemotherapy vs bevacizumab + chemotherapy. **Measurement Method:** Blinded Independent Central Review (BICR) using RECIST v1.1 criteria.

7.2 Secondary & Exploratory Endpoints - Overall Survival (OS). - Evaluation of safety, PK profile, and immunogenicity of pumitamidg. - Changes in cancer-related symptoms and Quality of Life (QOL). - Evaluation of candidate biomarkers associated with efficacy and safety.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring at each treatment cycle visit via laboratory tests, physical examinations, and patient reporting. **Serious Adverse Events (SAEs):** Required reporting to the Sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) established to review ongoing safety and efficacy data throughout the Phase 2/3 transition and beyond.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analyses (OR, PFS, OS); Safety Analysis Set for all patients receiving at least one dose of study drug. **Sample Size Justification:** $N = 990$, statistically powered ($\alpha = 0.05$, dual-sided) to detect a clinically meaningful difference in OR (Phase 2) and PFS/OS (Phase 3) between the experimental and active comparator arms. **Handling of Missing Data:** Subjects without documented progression or death will be right-censored at the time of their last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure protocol adherence, drug accountability, and source data verification. **Audit Trail:** Utilizing a validated Electronic Data Capture (EDC) system with comprehensive eCRF locking and audit trail tracking for all queries and data modifications.

11. Study Identifiers & Governance

Primary ID: {{CA266-0003}} **Registry Number:** {{NCT07221357}}

Sponsor/Lead Organization: {{Bristol Myers Squibb}} **Study Title:** {{A Study to Evaluate the Safety and Efficacy of Pumitamid in Combination With Chemotherapy Versus Bevacizumab in Combination With Chemotherapy in Participants With Previously Untreated, Unresectable, or Metastatic Colorectal Cancer}} **Official**

Regulatory Title: {{ROSETTA CRC-203: A Blinded, Randomized Phase 2/3 Study of Pumitamid in Combination With Chemotherapy Versus Bevacizumab in Combination With Chemotherapy in Participants With Previously Untreated, Unresectable, or Metastatic Colorectal Cancer}}

12. Clinical Framework (Colorectal Cancer Specific)

Primary Condition: {{Untreated, Unresectable, or Metastatic Colorectal Cancer}} **Diagnosis Threshold:** {{Histologically confirmed recurrent or metastatic colorectal adenocarcinoma, dMMR/MSI-H negative, BRAF V600E negative}} **Medical Methodology:** {{First-line (1L) systemic combination therapy}} **Optimization Target:** {{Objective Response (OR) and Progression-Free Survival (PFS)}}

13. Study Procedures & Methodology

Management Pathway: {{Standard oncology clinical pathway for 1L mCRC}} **Education Protocol (HCP):** {{Site Initiation Visits (SIVs), RECIST v1.1 training, and safety reporting protocols}} **Education Protocol (Patient):** {{Informed consent process, treatment schedule guidance, and symptom tracking}} **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) for tumor assessment and IDMC safety reviews}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 5 years monitoring period}} **Onsite Visit Cadence:** {{Aligned with 14-day or 21-day chemotherapy}}

administration cycles}} **Remote Monitoring Frequency:** {{Survival follow-up every 12 weeks (Q12W) post-treatment}} **Checklist Review Interval:** {{Tumor imaging typically every 8 weeks (Q8W) for efficacy evaluation}}

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				